

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT IDENTIFICATION****

Name: Patient 37

Age: 77 years old | Sex: Male

MRN: 000-TEST-37

Admission Date: 06/10/2026 | Discharge Date: 06/15/2026

Facility: Metro Medical Center, Orthopedic Surgery Unit

****PRIMARY DIAGNOSIS****

Severe osteoarthritis, right hip — s/p total hip arthroplasty (THA) with cemented prosthesis (06/11/2026)

****SECONDARY DIAGNOSES****

- Chronic microcytic anemia (baseline Hgb 9.8 g/dL, stable)
- Chronic pain syndrome
- Hypertension (controlled)
- Type 2 diabetes mellitus (non-insulin dependent)
- History of GERD

****HOSPITAL COURSE****

Patient 37 was admitted 06/10/2026 for elective right THA. Pre-operative labs and imaging unremarkable. Patient underwent cemented THA on 06/11/2026 under spinal anesthesia; operative time 78 minutes, EBL ~300 mL. Immediate post-op course uncomplicated. Patient tolerated advancement to TTWB (toe-touch weight bearing) on POD #1 with PT supervision. Wound healing progressing well, no signs of infection or hematoma. Foley catheter discontinued POD #2 without difficulty. Patient ambulated 80 feet with walker and contact

guard assist by POD #3. Pain controlled with multimodal regimen. No DVT symptoms (negative Doppler U/S bilateral lower extremities POD #2). Discharged to home with family support and home health PT/OT arranged.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Acetaminophen	650 mg	TID	10 days then PRN
Oxycodone	5 mg	q.4–6h PRN	2 weeks
Enoxaparin (Lovenox)	40 mg	SC daily	14 days (DVT prophylaxis)
Metoprolol tartrate	25 mg	BID	ongoing
Metformin	500 mg	BID	ongoing
Omeprazole	20 mg	q.d.	ongoing
Iron sulfate	325 mg	q.d.	ongoing

****WEIGHT-BEARING & ACTIVITY STATUS****

****TTWB (Toe-Touch Weight Bearing) right hip**** — do NOT progress beyond this without PT clearance. Advance ambulation as tolerated with assistive device (walker recommended for 4 weeks minimum). NO stairs for 2 weeks. Sleep with abduction pillow between knees × 8 weeks.

****HIP PRECAUTIONS — CRITICAL****

- NO hip flexion >90 degrees (no bending at waist; keep hips higher than knees when sitting)

- NO internal rotation (keep knee and toes pointing outward)
- NO crossing legs
- Use elevated toilet seat, use sock aid & reacher for ADLs
- Patient to wear hip precaution brochure on wrist band for 8 weeks

****WOUND CARE****

Incision clean, dry, intact. Steri-strips may be removed after 5 days. Keep dry × 3 weeks. Assess daily for erythema, drainage, warmth, or dehiscence. No submersion baths until cleared by surgeon (target ~6 weeks post-op).

****PHYSICAL & OCCUPATIONAL THERAPY ORDERS****

- PT: 3×/week × 4 weeks, then re-